INTRAOPERATIVE NEUROPHYSIOLOGY

Print Time Stamp: 06-05-2023 08:02 PM EST

Surgery Date: **04/12/2023** Medical Rec. #: 203589

Patient: **RODNEY C THOMPSON** **DOB:** 05/21/1973 **Age:** 49 **Sex:** Male

Surgeon: Michael Merrick, M.D.

Post Baseline 15:22

Final Trace Time: 16:45

Anesthesiologist:

Total Professional Time: 01:23

PID: 2691232

IONM Technologist: Rachel Flores

Location: BS&W TEXAS SPINE & JOINT HOSPITAL

Diagnosis: LUMBAR STENOSIS WITHOUT NEUROGENIC CLAUDICATION

Procedure: TLIF, PDF L4 - L5.

Conditions of the Recording:

Conditions of the Recording: All studies were performed on the aforementioned patient under real-time physician direct supervision via internet communication allowing continuous or immediate contact between the interpreting neurologist and surgeon. Please see tech notes for details of stimulation and recording.

Description of the Recording:

Somatosensory Evoked Potentials (SSEPs) were performed to monitor the sensory system by stimulating nerves in the upper and lower extremities. Baseline responses were recorded prior to the start of the procedure and subsequent responses were compared to baseline.

Screw tests were satsfactory

Lower extremity Free running Electromyography (EMG) was performed to monitor the integrity of the motor system and for nerve/root irritability. Recording electrodes were placed in muscles appropriate to the site of the procedure.

Summary:

SSEP EMG and screw test data were satisfactory

Impression:

Impression: This intraoperative monitoring study was unremarkable, as described above.

Thomas Trese, D.O.

NOTE: This report was signed via Electronic Signature by Thomas Trese, D.O. on 06/05/2023 04:03 PM

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Screening
None recorded.

Clinical Documents

OPERATIVE REPORT - 04/12/2023

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Texas Spine And Joint Hospital
                    OP REPORT - WWW20637186
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Account#: 826781                    Date of Procedure: 2023-04-12
Patient Name: THOMPSON, RODNEY CHARLES
MRN: 203589
Sex: Male
DOB: 05/21/1973
Proceduralist/Surgeon: MICHAEL MERRICK
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PRE-OPERATIVE DIAGNOSES:
  Spinal stenosis of lumbar region
    Lumbar region; With neurogenic claudication

  Lumbar radiculopathy
    Nerve Root: L4; L5

POST-OPERATIVE DIAGNOSES:
  Spinal stenosis of lumbar region
    Lumbar region; With neurogenic claudication

  Lumbar radiculopathy
    Nerve Root: L4; L5

PROCEDURES PERFORMED:
  L4/5 posterolateral fusion with TLIF

  L4/5 interbody cage placement

  L4 - 5 posterior instrumentation

  Local autograft

  DBM

  L4/5 posterior decompression bilaterally - this was beyond what was
  needed for the TLIF as the TLIF was done on the right but the
  decompression was done bilaterally.

ASSISTANTS:
  Dean Pennington, PA-C

ESTIMATED BLOOD LOSS:
  200mL

FINDINGS:
  See Post-Operative Diagnoses

SPECIMEN REMOVED:
  None

COMPLICATIONS:
  None

ANESTHESIA:
  General endotracheal tube

IMPLANTS:
  Alphatec

INDICATIONS:
  The patient is an 49 year old male who has a history of chronic back
  pain, bilateral leg pain and signs and symptoms of neurogenic
  claudication. The patient felt like he had failed conservative
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measures. After the risks and benefits of surgery were explained, he wished to proceed.

TECHNIQUE:

CONDITION:

Stable.

PROCEDURE IN DETAIL:

The patient was brought to the operative suite and anesthetized and intubated while supine on the hospital bed. He was then placed in a prone position on a Jackson table. All bony prominences were well-padded. The posterior lumbar spine was then prepped and draped in sterile fashion.

Prior to incision, a time-out procedure was performed. We identified the correct patient, the correct procedure, the correct operative level, and we confirmed that preoperative antibiotics had been given.

Next, a spinal needle was used with lateral x-ray to localize our incision. Next, an incision was made in the midline from the L4 to the L5 level. Dissection was carried down sharply through the skin with a scalpel and then with Bovie electrocautery down to the level of the fascia. The fascia was divided over the spinous processes, down over the lamina, out over the facets, and out to the transverse processes and sacral ala. We confirmed our level with lateral x-ray and a Kocher.

Next, pedicle screws were placed at L4 and L5 bilaterally in the following fashion: a rongeur was used followed by a drill, then gear shift, then ball-tip probe, then tapped, then ball-tip probe. Then the transverse processes were decorticated. The screws were then placed. After all screws were placed, we stimulated the screws. All screws stimulated above 20. We took AP and lateral x-rays and were satisfied with their position.

Next, attention was placed to our decompressions. We took down the spinous process and lamina of L4 up to the insertion point of the ligamentum flavum. We resected the top of the spinous process and lamina of L5. Foraminotomies were completed bilaterally until a Murphy ball probe was easily passed. On the right hand side at L4-5 we had to remove the entire facet in order to adequately decompress the foramen. Once we were done, we had excellent visualization of the dura. There was no longer central or foraminal stenosis. We confirmed we had completely decompressed the L4 and L5 nerves bilaterally. Of note, the decompression was beyond what we needed for the TLIF as the TLIF was done on the right but the decompression was needed and done bilaterally.

Next on the right-hand side at L4-5 we mobilized the dura. An annulotomy was made with an 11-blade scalpel. Discectomy was performed with shavers and curettes. Once discectomy was complete, we used a trial and selected our interbody cage. We placed local bone autograft and demineralized bone matrix within the cage and then tamped our cage into position. The TLIF portion of the surgery constituted the interbody fusion, which was separate from the posterolateral fusion. We then placed rods and set screws and final tightened them after compression was applied. We took final AP and lateral x-rays and were satisfied with all of our hardware positioning.

We then thoroughly irrigated the wound. We placed the remaining local bone autograft and demineralized bone matrix into the posterolateral gutters bilaterally for our posterolateral fusion at L4-5. The posterolateral fusion was separate from the interbody fusion.

We placed a 15 French-Blake drain deep to the fascia. The fascia was closed with interrupted 0 Vicryl sutures. The subcutaneous tissue was closed with interrupted 2-0 Vicryl sutures. The skin was closed with 4-0 Monocryl. Dermabond was applied to the skin incision. The drain was sutured in place.

All counts were correct at the end of the case.

The patient tolerated the procedure well without any complications and

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was transferred to recovery.

RISK FACTORS / PROBLEM LIST
BODY MASS INDEX:
BMI: 24.6
Height: 172.72cm
Weight: 73.39kg

Electronically signed by MICHAEL MERRICK MD on 04/12/2023 16:44 CDT

WWW20637186.1 by MICHAEL MERRICK MD, 04/12/2023 16:44 CDT

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